Fax cover sheet

Harbor Crest Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Keith Yamamoto, LCSW
Phone: 555-0141
Date: 10/05/2026 11:20
Pages (including cover): 12
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

54M MSSA bacteremia from L foot cellulitis, on IV cefazolin via PICC x 4 more weeks. Home infusion arranged per referral form. Hoping for d/c 10/06/2026.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Harbor Crest Medical Center (fictional)
Dept/Floor: 6 East Medicine
SW/CM/RN: Keith Yamamoto, LCSW
Phone/Pager #: 555-0141
Email: kyamamoto@harborcrest.example
Nursing Station #: 555-0142
Authorized by: Dr. P. Adeyemi
Phone #:

Patient Information

Patient Name: Luis Doe
MRN: HC-5027719
Date of Birth (dd/mm/yyy): 06/30/1972
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other
Ethnicity: Hispanic or Latino
Last 4 # of Social Security: xxx-xx- 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: Spanish / English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
MSSA bacteremia, L foot cellulitis
Admit Date: 09/30/2026
Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No
If no, will Home Health be ordered? ☒ Yes ☐ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No
If yes, Date: 09/30/2026
Results: CXR clear

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* ✕No	Active Infection	✕Yes* No
*Explain		*Explain	MSSA bacteremia - IV cefazolin via PICC
Self Administer All Meds	✕Yes No*	Ambulatory	✕Yes No
Continent of Bowel & Bladder	✕Yes No	Assistive Device	Yes* ✕No
Colostomy/Ileostomy	Yes ✕No	*Explain	
Foley Catheter	Yes ✕No	Hospice	Yes* ✕No
Diabetes	Yes ✕No	*Explain	
Insulin	Yes ✕No	Car	Yes ✕No
Diabetes related Oral Meds	Yes ✕No	Service Animal	Yes ✕No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ✕No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant
☐ Forgetful
☐ Cognitive Impairment
☐ Registered Sex Offender
☐ Other Explain

Length of Stay Authorized in Recupertive Care: 30 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS – AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) – Medi-Cal

Member

Member Name: Luis Doe

DOB: 06/30/1972

Member ID: OCH-48816352

Referring Provider

Facility: Harbor Crest Medical Center (fictional)

Clinical

Primary Diagnosis: Sepsis due to methicillin susceptible Staphylococcus aureus

ICD-10: A41.01

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 10/06/2026

Requested Days: 30

Plan Use Only

Status: Pending review

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Patient face sheet

Harbor Crest Medical Center (fictional) - PATIENT FACE SHEET

Patient: Luis Doe
MRN: HC-5027719
Account #: 99027719
DOB: 06/30/1972
Sex: Male
Preferred Language: Spanish / English
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 09/30/2026 14:45
Admit Type: Inpatient
Unit/Bed: 6E-604
Attending: Priya Adeyemi, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical - page 1

HISTORY AND PHYSICAL

Date: 09/30/2026 16:30

CC

L foot redness and swelling x 5 days, fever and chills

HPI

54 y/o M, unhoused (sleeping at a bus shelter), with blisters on L foot from ill-fitting shoes that became red, swollen and painful over 5 days. Fevers and rigors since last night. Denies IVDU.

ED: T 102.9F, HR 118, BP 108/64, WBC 19.1. L dorsal foot erythema to ankle, no open wound, no fluctuance.

XR L foot: soft tissue swelling, no bone erosion, no gas.

PMH

- Hypertension (not on meds for 1 year)

PSH

- None

Meds PTA

None

Allergies

NKDA

Social

Unhoused x 2 years. Tobacco 1 ppd. Denies EtOH or drug use. UDS negative.

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History and Physical - page 2

Exam

Gen: ill-appearing, AOx4

CV: tachycardic, no murmur. Lungs: clear

L foot: dorsal erythema and warmth extending to ankle, tender, intact skin, healing blisters, no drainage

Assessment/Plan

L foot cellulitis with sepsis.

1. Blood cultures x2, then vancomycin + cefazolin IV
2. MRI L foot to evaluate for deep infection / osteomyelitis
3. MRSA nares screen
4. Dispo: unhoused, CM consult for recuperative care

Signed: Priya Adeyemi, MD 09/30/2026 17:20

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Infectious disease consult

INFECTIOUS DISEASE CONSULT

Date: 10/02/2026 10:15

Reason: MSSA bacteremia

Blood cultures 09/30: MSSA 2 of 2 bottles. Repeat blood cultures 10/01 and 10/02: no growth to date.

MRI L foot 10/01: cellulitis of dorsal foot, no abscess, no osteomyelitis.

TTE 10/02: no vegetation, EF 60%.

Recommendations

- Stop vancomycin. Cefazolin 2 g IV every 8 hours.
- Complicated MSSA bacteremia (fever >72h before clearance): total 4 more weeks IV cefazolin via PICC, end date 11/03/2026.
- PICC placement once repeat cultures negative x 48h.
- Home infusion and home health nursing needed for PICC care and weekly labs (CBC, BMP).
- ID clinic follow up 2 weeks after discharge.

Signed: R. Marsh, MD (Infectious Disease)

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#6

Date: 10/05/2026 08:00

S: Foot pain 2/10. No fevers. Eating well.

O: T 98.5F HR 78 BP 128/80 RR 16 SpO2 98% RA. AOx4.

L foot: erythema resolved to small area over dorsum, no warmth.

A/P: MSSA bacteremia from L foot cellulitis, cleared.

- Afebrile since 10/03
- Cefazolin 2 g IV every 8 hours via R arm PICC (placed 10/04) through 11/03/2026
- PICC tip confirmed SVC on CXR 10/04
- Amlodipine 5 mg PO daily started 10/03 for HTN, BP controlled
- Dispo: medically ready once home infusion / home health confirmed

Signed: Priya Adeyemi, MD

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Nursing note

NURSING NOTE

Date: 10/05/2026 06:50

Pt AOx4, cooperative.

Pt up in room and to bathroom independently, no device.

Showered independently with PICC cover. Toileting independently, continent.

R arm PICC: dressing clean, dry, intact; flushes without difficulty.

Pt self-administered amlodipine 5 mg PO at 06:00 with RN observation; states purpose and schedule.

Signed: S. Delgado, RN

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Case management note

CASE MANAGEMENT NOTE

Date: 10/05/2026 10:30

Pt unhoused, agreeable to recuperative care.

Home infusion referral sent 10/03 to Seaside Infusion Services (fictional) 555-0144 for cefazolin and PICC care.

Home health nursing referral sent 10/03 to Valley Bloom Home Health (fictional) 555-0188 for weekly PICC dressing change and labs.

Status 10/05: neither agency has accepted yet. Awaiting insurance authorization review.

Self-administration teaching cannot start until infusion agency accepts.

ID clinic follow up to be scheduled after discharge date set.

Signed: Keith Yamamoto, LCSW

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Vitals, labs and microbiology

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/03 20:00	98.9	84	130/82	16	98%	RA
10/04 08:00	98.4	80	126/78	16	98%	RA
10/04 16:00	98.7	82	132/80	16	97%	RA
10/04 23:00	98.2	76	124/76	16	98%	RA
10/05 08:00	98.5	78	128/80	16	98%	RA

LABS

09/30 | WBC 19.1 H | Hgb 13.4 | Plt 266 | Na 135 | K 3.9 | Cr 1.1 | Glucose 112
10/04 | WBC 9.0 | Cr 0.9

MICROBIOLOGY

09/30 Blood cultures x2: Staphylococcus aureus, methicillin susceptible (MSSA), 2 of 2
10/01 Blood cultures x2: no growth to date
10/02 Blood cultures x2: no growth to date
09/30 MRSA nares PCR: Negative
SARS-CoV-2 swab: to be collected on day of discharge (not yet done)

IMAGING

09/30 CXR: no acute process
10/01 MRI L foot: cellulitis, no abscess, no osteomyelitis
10/04 CXR: R PICC tip in SVC

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Discharge medication list (draft)

DISCHARGE MEDICATION LIST - DRAFT

Patient: Luis Doe MRN: HC-5027719

- Cefazolin 2 g IV every 8 hours via PICC through 11/03/2026 (home infusion - pending agency acceptance)
- Amlodipine 5 mg PO daily
- Acetaminophen 650 mg PO every 6 hours PRN pain
- Nicotine patch 21 mg/24h daily

Lines

- R arm PICC (placed 10/04/2026) - remove at end of therapy

Follow up

- ID clinic 2 weeks after discharge (to be scheduled)

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